



Eternal Life Hospice, Inc.

Branding & Marketing *Brief.*

*An expedited roadmap — built
for urgency without compromise.*

TRACK ONE · REBRAND & MARKETING

Urgency, *held with care.*

Speed and quality are not opposites. They share the same parent — clarity. When the brand is decided, the timeline is short. When the brand is still being negotiated mid-build, every week becomes a month. We are working from a decided brand.

Eternal Life Hospice is in an unusual position. The strategic foundation is locked — voice, three differentiators, three service pillars, the phrase bank, and the central platform, *The Eternal Standard*. A working visual direction is in market, carried by a presentation folder and three rack cards now in revision. The visual system itself — taken as a whole — is pending formal exercise. What remains in front of us is execution: bringing every surface (digital, print, verbal, referral) into parity with what's been decided, and finishing the system around what's already working. The work ahead is layered on top of what's already working beautifully — a founder who has grown census from two to one hundred, a clinical reputation that walks into rooms before she does, and a partner network that already trusts her by name.

This brief covers Track One — rebrand and marketing — over the next forty-five days, with a horizon of six months. Track Two, public relations, follows on its own schedule in a separate plan, and is sequenced to begin only when the operational and message foundation is stable. Every surface produced in Track One is built so that Track Two can layer on top of it later without rework.

The non-negotiable sequence

Clarify first. Build trust second. Align the team third. Soft launch fourth. Only then move into broader referral outreach, content marketing, and — eventually, on its own timeline — public relations. This sequence is what permits urgency. Skipped, it produces beautiful materials that families and referral partners cannot trust. Honored, it produces a fast launch that holds up under scrutiny.

A NOTE ON TEMPO

An expedited timeline is not a compressed one. It is one in which nothing waits. Approvals, not production, are the most common bottleneck in brand work of this kind. The cadence proposed here protects against that — short approval windows, single decision-makers per surface, a daily check-in for the first fourteen days, and pre-staged production so each green light moves immediately into the world.

Where the brand stands today

What's locked, *what's in motion.*

Every surface forward ladders back to what's locked. What's in motion lands before the next round of production.

Locked

VOICE & TIER

Calm. Refined. Clinically credible. Warm without sentimentality. Human and direct; never vague about what happens next. Where peers use scale-and-protocol language, Eternal Life uses presence-and-attention language. Tier read: premium, warm, organic — luxury felt, never claimed.

THREE DIFFERENTIATORS (DESIGNED TO TRAVEL)

Founder-led

Decisions are made by the person whose name is on the door. Care does not get lost in committee.

Independent

Not part of a hospital system or national chain. Census growth is earned in the field, one family at a time.

Integrative

Eight modalities woven into clinical care: music, massage, reiki, aromatherapy, pet, audiology, holistic medicine, doula.

THREE SERVICE PILLARS & PHRASE BANK

Physician-Led. Hospice-certified physicians — foundational eligibility and credibility. **Respiratory.** Specialized comfort for COPD, lung disease, and end-stage cardiopulmonary patients. **Integrative Therapies.** Eight modalities, woven into the plan of care, not offered as extras. "Care That Honors Life" (emotional promise) · "We Are Here. Always." (reassurance promise) · "The First 72 Hours" (family and referral-partner guide language) · "Compliance is patient protection" (professional-facing message).

THE ETERNAL STANDARD — CENTRAL BRAND PLATFORM

"Patients and families deserve dignity, clarity, comfort, and protection during one of life's most vulnerable moments." Six pillars: dignity, clinical confidence, family guidance, whole-person comfort, responsive presence, compliance-conscious care. All rebrand, web, social, referral, and PR materials ladder back to this platform. Not a certification — the promise the brand makes.

In motion

A working visual direction is in market — a presentation folder and three rack cards built around an editorial serif logotype with the infinity mark over *Eternal*. These carry the strategic foundation well, and Aleksandra has confirmed the direction. They are being revised: copy first, to bring all six pillars of The Eternal Standard forward through the welcome letter and pillar spreads; logo system second, to build the full set of lockups every surface needs. The visual system taken to formal exercise sits within Movement Two.

The two-track framework

Two tracks, one foundation.

The work is sequenced into two parallel tracks, run on different cadences. Track One is the subject of this brief. Track Two follows in a separate plan, on its own schedule, when the foundation is stable.

TRACK ONE

Rebrand & Marketing

The Eternal Standard becomes the organizing promise behind every front-facing piece. Folder, rack cards, website, social, email, referral partner materials, and intake scripts are revised — not thrown away, refined — so the message becomes intentional, consistent, and elevated.

Days one through forty-five. Soft launch by day twenty-one. Referral-ready by day thirty. Active referral cadence from day thirty forward.

THIS BRIEF

TRACK TWO

Public Relations

Eternal Life's authority is built carefully and in stages. The path begins with entity clarification and message alignment, builds through a complete media kit and referral partner reassurance, soft-launches into owned channels, and then moves outward — local, regional, professional, and only eventually national.

Sequenced as a separate plan. Begins only when Track One's foundation is stable and the operational and message layers are clean.

SEPARATE PLAN

Why the tracks run separately

Public relations placed too early in a rebrand creates exposure where there should be foundation. A reporter calls; the website is half-updated; a referral partner hears one message and reads another. The remedy is sequence, not slowdown. Track One produces a coherent, signed-off body of work — the website, the collateral, the team scripts, the referral packet, the family pathway — that Track Two then activates outward through earned media, speaking, and trade press.

How Track One is built to enable Track Two

Every Track One deliverable is built to do double duty. The founder bio is institutional enough to hand a reporter. The fact sheet is the seed of a press kit. The Eternal Standard one-pager is both a referral leave-behind and a media reference document. The First 72 Hours guide is both a family handout and a Track Two story angle. Nothing in Track One has to be redone for Track Two — only extended, contextualized, and pitched.

THE TRACK TWO PLAN

Track Two is documented in a separate brief. The nine-step PR path (Clarification → Message Alignment → Marketing Kit → Media Kit → Referral Reassurance → Soft Launch → Local → Professional → National) governs its sequence — and does not begin before Track One's foundation is stable.

The rules that govern every surface

Three rules, *held together*.

Internalized correctly, these rules make most editorial decisions for us. Every page, every line of copy, every photograph should pass all three before it goes to print or live.

Rule One — *The story transfers. The face does not.*

Aleksandra's story is brand equity. The grandmother lost on hospice. The open-heart survival. The decade in the field. The immigration story that ends in this work. That story carries weight, and it carries forward. Her *face* — present on every page, oversized, narrating — does not. It signals founder dependency, and founder dependency depresses what a thoughtful evaluator is willing to pay for a business. Census growth currently *requires* her in the field. That is the right operational decision. It is also the reason her face should stay *small* on the brand surfaces an outsider will use to form a first impression.

Rule Two — *Aleksandra is the standard-bearer.*

Never the victim. Never the fighter. Never the hospice hero. The stronger story is the founder who stayed focused on patient dignity, continuity of care, family protection, and the future of end-of-life care. Standard-bearer is a position of quiet authority, not crusade. It positions Aleksandra above the noise — credible to a hospital case manager and a journalist alike — without ever asking the brand to perform indignation, sentimentality, or self-promotion on her behalf.

Rule Three — *Two agency stories. One bridge.*

Westlake Village Hospice is the restrained, legal-reviewed regulatory story — operational, anchor agency, held in measured language. Eternal Life Hospice is the forward-facing brand — the new platform, the rebrand, the story the public encounters. Aleksandra is the bridge between them: the same standard-bearer behind both agencies, the same dignity-centered approach, the same compliance-conscious care. The two stories travel together when she chooses, separately when she doesn't. Neither contradicts the other.

The story transfers. The face does not. The founder is the standard-bearer. Two agencies, one bridge. Every page passes all three.

THE THREE RULES · BRIEF TEST

A NOTE ON THE DUAL SURFACE

In the field, Aleksandra remains the face. Referral partners hand out her cell, walk the flyers, sit at the bedside. Nothing changes there. The dual surface lets the brand read institutional *and* the founder remain irreplaceable, in two separate channels, at the same time. The brand surfaces are the ones a stranger forms a first impression from. Those stay institutional. The field stays personal.

From day one

What moves *today*.

Day One is a command-center session. Six clearance items, none of them creative. All of them prevent wasted design, duplicate copywriting, and public-facing confusion later.

Day One — command center clearance

Six items, before any design begins.

1. Confirm public identity. Official public brand name (Eternal Life, Eternal Life Hospice, or another approved presentation). Primary domain confirmed; ownership of any duplicate or outdated web presence resolved. Official address, service area, primary phone, and public contact email confirmed.
2. Reclaim digital ownership. Remove ghost administrators from the WordPress instance (Jeff Cecil, iFlint). Confirm Aleksandra is sole super-admin. Document recovery email, two-factor method, registrar and hosting credentials in a single secured note.
3. Verify the claims layer. Licensing, accreditation, certifications, association logos, and the exact language permitted publicly. Separate operationally true services from aspirational or partner-dependent ones. Approve the level of public emphasis around End of Life Option Act / medical aid-in-dying support.
4. Audit the vendor stack. Pull the Codexal/SaaS BizHub agreement (signed Nov 6, 2025) and confirm the November 6, 2026 termination date in writing. No auto-renewal.
5. Set the approval lane. Aleksandra is single decision-maker on brand surfaces. Conduit holds the pen. No third party reviews creative output. The compliance/clinical/legal approval chain is named for medical and regulatory language. Written down, shared once.
6. Open the file. A single working folder, named, shared, ready to hold every deliverable from this point forward — alongside a decision log (one line per decision, dated, signed off).

RED FLAGS TO CLEAR BEFORE ANY LAUNCH

Any inconsistent address or location reference. Any duplicate website. Any outdated footer date, typo, or consumer-grade email. Any logo, badge, or accreditation reference not explained clearly. Any service claim the team cannot consistently fulfill. Any promise around response time, same-day admission, or specialty care not operationally approved.

Workstreams & owners

Eight workstreams, *running in parallel.*

An expedited timeline depends on work happening in parallel, not in sequence. Each workstream has one responsible owner and one final approver. Without that structure, the rebrand slows down or becomes inconsistent.

WORKSTREAM	RESPONSIBLE	APPROVER	RISK IF MISSED
<i>Brand Strategy</i>	Conduit	Aleksandra	Mixed messages; weak differentiation; the rebrand reads as a refresh, not a position.
<i>Compliance & Claims</i>	Admin / clinical lead	Legal / clinical	Public trust damage or regulatory exposure. Claims unsupported by operational reality.
<i>Website</i>	Web lead (Conduit-managed)	Aleksandra	Traffic arrives but doesn't convert. Families and partners cannot find what they came for.
<i>Referral Partner Materials</i>	Conduit	Aleksandra / clinical	Referral partners do not understand why Eternal Life is different from peers.
<i>Family Materials</i>	Conduit	Clinical / Aleksandra	Families remain confused, overwhelmed, and slow to call.
<i>Internal Scripts & Intake</i>	Operations lead	Aleksandra	Team gives inconsistent answers. The rebrand loses credibility on the first call.
<i>CRM & Pipeline</i>	CRM owner	Operations / Aleksandra	Leads fall through cracks. Referral activity untracked. Follow-up by memory only.
<i>Photography & Print</i>	Conduit	Aleksandra	Inconsistent visual tier; rack cards and folder feel separate from the website.

Public Relations is its own workstream, run on a separate timeline in a separate plan. Mentioned here only so it is not assumed to be part of Track One's parallel build.

The expedited timeline

Forty-five days, in five movements.

Soft launch by day twenty-one. Referral-ready by day thirty. Active referral cadence from day thirty forward. Each movement compounds the last. Foundation makes asset production fast; asset production makes the website sprint fast; the website sprint makes soft launch possible.

Movement One

DAYS 1-7 ·
FOUNDATION &
TRUST CLEANUP

Clearance, cleanup, and command center.

The six Day One items, completed. Google profile, listings, duplicate site assets, old footer dates, consumer-grade emails, and visible credibility friction cleaned. Workstream owners named. Daily war room begins.

- Days 1-2: identity, claims, approvals, and red flags resolved
- Days 3-5: trust cleanup across Google, listings, email, bios, public directories
- Days 4-7: message architecture drafted — Eternal Standard one-pager, First 72 Hours framework, family pathway, referral message

Movement Two

DAYS 8-14 ·
MESSAGE
REFINEMENT &
VISUAL SYSTEM
BUILD

The message gets revised; the system gets finished.

Copy on the welcome letter, three-pillar spread, and rack cards revised to bring all six pillars of The Eternal Standard forward. Logo system built out — primary, horizontal, mark-only, knockout, single-color, with clear-space and minimum-size rules. Visual direction taken to formal exercise. One-page brand guide produced. Folder + three rack cards revised toward print-ready. Photography style sheet locked.

- Copy revisions to existing materials: welcome letter, pillar descriptions, rack-card framing
- Logo system complete and archived (all lockups, color modes, file formats)
- Brand guide v1 — one page, never longer
- Folder + three rack cards: revised, signed off, print-ready files released
- Photography style sheet: care, place, hands, light. No staged smiles.

Movement Three

DAYS 15-21 ·
WEBSITE &
COLLATERAL SPRINT
→ SOFT LAUNCH

Build, populate, soft launch.

Website built — Home, The First 72 Hours, For Families, For Referral Partners, Services, About / Leadership, Contact / Evaluation. Core collateral produced: referral one-sheet, Family Peace Guide outline, Hospice Decision Navigator, folder / card system. Email signatures and social profile copy unified. Print orders placed by day eighteen so collateral arrives the same week launch goes live.

- Site populated and in staging by day eighteen
- SEO foundation: titles, descriptions, schema, sitemap
- Core collateral signed off and at the printer by day eighteen
- Soft launch by day twenty-one — trusted advisors, internal team, friendly referral contacts
- Feedback collected; confusion, missing proof, and risky language flagged for revision

*Movement**Four*

DAYS 22-28 ·
INTERNAL
ALIGNMENT &
REFINEMENT

The team is the launch.

Soft launch feedback integrated. Phone scripts (family inquiry + referral partner) approved. Intake checklist. First 72 Hours checklist. Claims boundary sheet. End of Life Option Act language guide (legally reviewed). Escalation process for urgent clinical questions. CRM pipeline configured and live. Site corrections completed. Materials approved as referral-ready by day twenty-eight.

- Phone scripts and intake checklist trained across the team
- Claims boundary sheet: what the team may and may not say
- CRM live with Family Inquiry, Referral Partner, and Reputation pipelines
- Soft launch corrections completed; materials approved referral-ready
- Referral outreach list finalized with named contacts

Movement Five

DAYS 29-45 ·
REFERRAL
ACTIVATION

The brand enters the world.

Warm referral outreach begins — hospitals, oncology, PCPs, case managers, SNF/ALF/RCFE administrators, geriatric care managers, elder law attorneys, home health providers. Aleksandra walks the folder + three rack cards personally where the relationship warrants it. The first modality campaign deploys (aromatherapy or music therapy — recommended order on the horizon page). Video testimonial production begins, one family at a time, captured in long form, edited into short, dignified pieces. Inquiry and referral data flowing into the CRM.

- Warm referral outreach with approved packet — days 29 onward
- Facility / physician / discharge-planner conversations scheduled
- First modality campaign live — week of day 35
- First video testimonial in production — week of day 38
- Weekly inquiry/referral data review; friction points fixed inside seven days

WHAT THIS TIMELINE DOES NOT MEAN

It does not mean rushing medical or legal language. It does not mean launching ads before the website and intake process are ready. It does not mean chasing press before Eternal Life has a clean public presence — that is Track Two's territory, on Track Two's timeline. It does not mean putting every possible service online before verifying what is operationally true.

Cadence & gates

Cadence *and* gates.

Two structures hold the timeline. A short daily check-in for the acute build. Six approval gates that prevent the team from moving into public launch before the right items are signed off.

The daily war room — first fourteen days

Twenty minutes. Same time. Aleksandra and Conduit. Operational, not philosophical. Five questions, in this order. After day fourteen the war room dissolves into the weekly cadence (Governance, page seventeen).

- ☐ What was approved yesterday?
- ☐ What is blocked today?
- ☐ What public-facing information still needs verification?
- ☐ What copy or design item needs final sign-off?
- ☐ What could create trust friction if seen by a family or referral partner today?

The six approval gates

No surface goes public without passing the corresponding gate. Gates are sequential — Gate Two cannot be reached without Gate One, and so on.

GATE 1 *Identity*

Public name, primary domain, address, phone, email, service area, and ownership of any duplicate or outdated web assets. Approver: Aleksandra / Admin.

GATE 2 *Claims*

Licensing, accreditation, association logos, insurance language, physician language, specialty services, response-time promises, End of Life Option Act language. Approver: Compliance / Clinical / Legal.

GATE 3 *Messaging*

The Eternal Standard one-pager, taglines and phrase bank, family pathway, referral partner message, sensitive-topic boundaries. Approver: Aleksandra / Conduit.

GATE 4 *Operations*

Phone scripts (family + referral), intake checklist, First 72 Hours checklist, escalation process, CRM workflow live. Approver: Aleksandra / Operations.

GATE 5 *Soft Launch*

Website, forms, contact paths, collateral, team readiness, trusted-advisor feedback round completed and corrections integrated. Approver: Aleksandra.

GATE 6 *Referral Launch*

Referral page, one-sheet, outreach list, tracking process, follow-up cadence, modality campaign template ready. Approver: Aleksandra / Conduit.

The pages, *the leave-behinds*.

The website is built around trust and conversion, not just service descriptions. Families need to understand what happens after they call. Referral partners need a fast, professional way to refer. The collateral makes the same promise on paper that the website makes on screen.

Website structure

PAGE	PURPOSE	MUST INCLUDE
Home	State position clearly and quickly.	Headline, subhead, family CTA, referral CTA, Eternal Standard, link to The First 72 Hours.
The First 72 Hours	Own immediate onboarding.	Evaluation, eligibility review, care plan, equipment/medication coordination, family education, emergency protocol.
For Families	Reduce fear and confusion.	What hospice is, what happens after calling, common signs, what to expect, FAQ.
For Referral Partners	Make professional referrals easy.	Referral form, service area, response expectations, clinical differentiators, contact path.
Services	Organize without looking scattered.	Core hospice services, specialty care, whole-person support, integrative modalities. Verified items only.
About / Leadership	Humanize authority; institutional read.	Aleksandra bio (standard-bearer voice), small contextual portrait, signed letter, care philosophy, credentials.
Contact / Evaluation	Convert quickly.	Phone, form, after-hours guidance, referral path, privacy language.

Core collateral inventory

- ☐ **Referral partner one-sheet** — who Eternal Life serves, why Eternal Life is different, how to refer
- ☐ **The First 72 Hours handout** — calm, family-facing explanation of what happens immediately after admission
- ☐ **Family Peace Guide** — digital and printed; fear, breathing changes, pain, final-stage questions
- ☐ **Hospice Decision Navigator** — "Is It Time for Hospice?" checklist, web form and printed
- ☐ **Folder + three rack cards** — folder is the system, rack cards are the pillars
- ☐ **Aleksandra's signed letter** — inserted in the folder, signed in wine ink, small contextual portrait
- ☐ **Email signature + social profile copy** — unified across team and channels

By *workstream*

The checklists.

Each checklist is scoped to a workstream. None is exhaustive; all are sufficient. The shape of the work, not the length of the list, is what matters.

1 · Brand Identity Assets

- ☐ Primary logotype — color, single color, knockout, vertical, horizontal
- ☐ Infinity mark over *Eternal*, isolated for favicon and embroidery use
- ☐ One-page brand guide: palette hex/CMYK/Pantone, type pairings, spacing rules, do-and-don't
- ☐ Business cards (Aleksandra + clinical leadership), letterhead, envelope, email signature
- ☐ Photography style sheet — care, place, hands, light. No staged smiles.
- ☐ Logo file archive: EL-Logo-Master/ folder with sealed versions, last-updated date

2 · Digital Infrastructure

- ☐ WordPress super-admin reduced to one person; all ghost admins removed (Jeff Cecil, iFlint)
- ☐ Hosting credentials, registrar credentials, two-factor methods documented in single secured note
- ☐ Domain email confirmed; DNS records audited (MX, SPF, DKIM, DMARC)
- ☐ Analytics installed and configured (server-side preferred where possible)
- ☐ Codexal/SaaS BizHub contract: termination date confirmed in writing for November 6, 2026; no auto-renewal
- ☐ Backup of any existing site assets pulled and stored independently before any rebuild

3 · Internal Scripts & Intake Tools

- ☐ Phone script for family inquiries (calm, refined, clinically credible voice)
- ☐ Phone script for referral partners (efficient, respectful, fast to the point)
- ☐ Intake checklist — what is captured on every call, in order
- ☐ First 72 Hours checklist — what happens between first call and end of day three
- ☐ Approved services list — only operationally verified items
- ☐ Claims boundary sheet — what the team may and may not say publicly
- ☐ End of Life Option Act language guide, legally reviewed before any external use
- ☐ Escalation process for urgent clinical questions
- ☐ Review and testimonial request process, if and when compliant

4 · Photography & Portrait

- ☐ Shot list: hands, light through window, threshold of a door, a folded blanket, an unspoken moment between two people. No stock-photo poses.
- ☐ Aleksandra portrait session: one location, natural light, neutral background, three-quarter framing – institutional read, small final use size
- ☐ Photographer briefed on tier read (premium, warm, organic), the three rules, and dual-surface principle
- ☐ Model releases on file for any patient-family imagery (separate consent for marketing use)
- ☐ Final selects archived in three resolutions: web, print, presentation

5 · Print Collateral

- ☐ Folder: final cover edits, bleed/crop confirmed, paper stock specified (uncoated cream preferred)
- ☐ Three rack cards (one per pillar): copy laddered to The Eternal Standard, credentials placed with restraint
- ☐ Aleksandra's signed letter: inserted into folder, signed in wine ink, dated
- ☐ Portrait usage: small, contextual; never larger than 2.5 inches on any printed surface
- ☐ Print quantities sized for the named referral outreach list, plus 30% reserve
- ☐ One reorder vendor on file with SKUs noted for re-prints inside 48 hours

6 · Referral Partner Outreach

- ☐ Named outreach list: oncology, pulmonology, cardiology, neurology, geriatrics, internal medicine, hospital case managers, SNF/ALF/RCFE administrators, geriatric care managers, elder law attorneys, home health providers, funeral homes
- ☐ Tier one: warm contacts who already know Aleksandra by name. Tier two: cold but on-territory.
- ☐ Introduction script for each tier – written in Conduit voice, sized to a thirty-second read
- ☐ Folder + three rack cards as the leave-behind; never a single rack card alone
- ☐ Follow-up cadence: handwritten note within 48 hours of every in-person drop
- ☐ Modality cadence: one modality every six to eight weeks (see page 16 for recommended order)
- ☐ Tracking sheet for who has received which materials, last touch date, next action – captured in the CRM

7 · Compliance & Credibility

- ☐ CMS certification and CDPH licensure surfaced on About and footer – with restraint, never as a header
- ☐ Best of 2026 BusinessWalk recognition placed on About, not Home
- ☐ Insurance acceptance list rationalized across web, folder, and rack cards (one source of truth)
- ☐ Phone number consolidation: 805-953-7273 listed as primary across every surface
- ☐ Service area map: matched language across all surfaces (LA, Ventura, Orange counties)
- ☐ Privacy, HIPAA, and accessibility statements current and linked from the footer
- ☐ Every claim ("same-day," "physician-led," "24/7") confirmed against operational reality before going live

8 · CRM & Pipeline Readiness

- ☐ Family Inquiry pipeline configured (see page 14)
- ☐ Referral Partner pipeline configured
- ☐ Reputation / Experience pipeline configured
- ☐ Lead source tracking enabled on every inbound channel
- ☐ Automation: immediate thank-you, internal alert, what-happens-next email
- ☐ Weekly inquiry/referral data review on the agenda from day 29 forward

The operating layer

The CRM layer.

The rebrand only works if leads and referrals are handled cleanly. The CRM is the operating layer behind family inquiries, referral relationships, follow-up, and reputation. Without it, the most beautiful brand in the world loses its first call.

Three pipelines, configured before referral launch. Each has a sequence of stages and a small number of automations. The point is not sophistication. The point is that nothing is lost, no follow-up is forgotten, and Aleksandra knows on day thirty exactly what is in motion.

Family Inquiry

New inquiry → Contact attempted → Family reached → Evaluation scheduled → Eligibility review → Admission pending → Admitted / Not eligible / Follow-up later

Automations: immediate family thank-you, internal alert, what-happens-next email (links to The First 72 Hours), follow-up task if family not reached within 24 hours, Family Peace Guide delivery.

Referral Partner

New partner → Warm intro → Meeting requested → Meeting completed → Materials sent → First referral → Active partner → Follow-up needed

Automations: post-meeting thank-you, digital referral packet, monthly education send (modality campaign feed), referral confirmation, follow-up reminder at 30 / 60 / 90 days.

Reputation & Experience

Family check-in → Service feedback → Concern escalation → Testimonial request / do-not-request → Bereavement follow-up

Automations: careful-timing rules around testimonial requests (compliance-conscious), internal alert for any concern flagged, bereavement support reminder cadence.

Minimum tracking metrics

Reviewed weekly from day 29 forward. The point is not analytics — the point is finding friction before it costs a family.

- ☐ Source of every family inquiry and every professional referral
- ☐ Response time from inquiry/referral to first contact
- ☐ Evaluation scheduled, admission pending, admitted, not eligible, or follow-up
- ☐ Referral partner meetings completed and materials delivered
- ☐ Top family questions asked during intake (drives Family Peace Guide and FAQ updates)
- ☐ Website calls, form submissions, referral page visits, guide downloads
- ☐ Family experience feedback and any service-recovery needs

What to do, what to hold

The priority *matrix*.

Three lanes. Every action belongs to one. Attractive but premature work moves to lane two or three — never sneaks into lane one because it feels exciting.

Do immediately	Do after foundation	Hold for Track Two plan
Confirm public identity, address, claims, and approval chain.	Warm referral outreach to named tier-one and tier-two contacts.	National press pitches and consumer-business outlets.
Clean website, listings, contact, email signatures, and Google profile.	Website resource content (signs it may be time, what hospice is, family education).	Paid advertising to landing pages before intake is confirmed working.
Approve The Eternal Standard and the phrase bank.	Modality campaign rollout — one modality every 6–8 weeks.	Broad social campaigns with generic hospice messaging.
Build family-facing and referral-facing scripts.	Folder and leave-behind printing for tier-three outreach expansion.	Claims about services not operationally verified.
Create the First 72 Hours framework and handout.	Local podcast and community conversation opportunities (Track Two soft-launch territory).	Aggressive End of Life Option Act messaging without legal review.
Set up the CRM pipelines, alerts, and lead-source tracking.	Small Google Search Ads test — only after intake is confirmed working end to end.	Earned-media pitches, speaker bookings, awards submissions.
Remove ghost admins; reclaim digital ownership.	Video testimonial production, one family at a time.	Trade press placements and hospice industry features.

A NOTE ON LANE THREE

Lane three is not the discard pile. It is the work that becomes Track Two when the foundation is stable. Every item there has a place — just not yet. The Track Two plan picks them up in the right sequence: Clarification → Message Alignment → Marketing Kit → Media Kit → Referral Reassurance → Soft Launch → Local → Professional → National. Nothing in lane three is begun until Track One's six gates have passed.

The competitive frame

Where Eternal Life *stands*.

Three institutional peers anchor the local market: Mission, Buena Vista, and Silverado. They compete on scale, protocol, and continuity inside larger systems. Eternal Life does not compete with them on those terms — and should never attempt to.

What the peers do well

Mission, Buena Vista, and Silverado deliver care through scale. They are credible, predictable, and present in the same hospital corridors Eternal Life works in. A referral partner who hands a family one of their cards is making a safe choice — and safety, in this category, is real currency.

Where Eternal Life is different

The three locked differentiators do the work: **founder-led** (a real person is accountable), **independent** (no system politics, no corporate review delays), **integrative** (eight modalities, woven into the plan of care, not advertised as extras). The right family for Eternal Life is one for whom the institutional choice feels too distant and the boutique choice feels too uncertain. Eternal Life is the third option — boutique scale, institutional credibility.

Where peers compete on scale and protocol, Eternal Life competes on presence and attention. Both are credible. Only one is rare.

The reference template — Assisted Home Health

Assisted Home Health (AssistedCares.com), family-owned since 1989, is the closest model of how outcomes-language translates a clinical reality into a marketing reality. They lead with measurable claims: a thirty-two percent readmission reduction; clear comparison charts of hospice and palliative care; visualized care protocols.

Eternal Life has equivalent or stronger credentials. They have not yet been translated into outcomes language. That translation is one of the quiet wins of Movement Three — and one of the easiest. Most of the data exists. What's missing is the sentence that turns it into a number a referral partner can repeat.

A NOTE ON THE END OF LIFE OPTION ACT

Eternal Life's service inventory includes the End of Life Option Act. This is a differentiator that requires careful positioning. It is not led with. It is mentioned, where appropriate, alongside compliance and dignity language — never as a feature, always as part of the breadth of choice the brand protects for families. The language guide for it is legally reviewed before any external use.

The horizon

Beyond forty-five days.

Once the brand is in the world, the work changes shape. Less production, more cadence. Less invention, more iteration. The next horizon is about depth and proof — and about giving Track Two a clean foundation to build on.

The modality cadence

One modality introduction to the referral network every six to eight weeks. Each follows the same template, in the same voice, with the same visual treatment — so referral partners begin to anticipate them and recognize the brand by its rhythm. Recommended order:

Already live: Sound healing (deployed for WVH; the template Eternal Life adapts).

First Eternal Life modality: Aromatherapy (gentle, sensory, broadly understood). **Then:** Music therapy (strong emotional resonance, well-credentialed). **Then:** Guided meditation, pet therapy, reiki, massage, and legacy & life review — in that order, each spaced six to eight weeks apart.

The testimonial library

Video testimonials, captured one family at a time, in long form, edited into short and dignified pieces. The first by day forty. A second by day eighty. A third by day one-twenty. Three is enough to seed the website's About page, the referral folder's insert, and a six-week social rotation — and to give Track Two real material when it opens.

Acquisition-ready posture

Every surface produced from Movement One forward has been built to read institutional. By month four, the surfaces an outside evaluator would encounter — site, folder, press if it begins under Track Two, testimonials, referral network's verbal description — should describe a hospice with a remarkable founder, not a founder with a hospice attached. That is the posture. It is held quietly, not announced.

The hand-off to Track Two

Once the foundation is stable — gates passed, CRM working, soft-launch corrections integrated, modality cadence underway — Track Two opens. It begins with Clarification (which it inherits clean from Track One) and moves through Message Alignment, Marketing Kit Revision, Media Kit Development, Referral Reassurance, Soft Launch, Local Credibility, Professional Authority, and National Elevation. The Track Two plan documents the schedule, decisions, and assets. This brief simply notes the hand-off and stops there.

Governance & quick reference

How decisions *get made*.

Decision rights

Aleksandra is the single decision-maker on brand surfaces. Conduit holds the pen and presents options. No third-party reviewer participates in the creative chain. The compliance/clinical/legal approval chain is named for medical and regulatory language only.

Cadence

Days 1-14: daily war room, 20 minutes, five questions (page ten). Day 15 onward: one thirty-minute call per week, same day, same time. Written status note the day after, with three sections — *decided this week*, *shipped this week*, *needs your green light this week*. The third section never has more than five items. Nothing waits more than seven days for an answer.

The record

One working folder, one brand reference sheet (page three of this brief, kept current), one decision log. The decision log is short — one line per decision, the date, the decision-maker. It exists so that no question already answered is asked twice.

Thirty-day quick reference

The expedited timeline (pages 8-9) governs the work. This grid is a printable companion — a day-by-day at-a-glance for the working folder.

Day 1	Clarification meeting. Public name, domain, address, service area, contacts, approvals confirmed.	Day 2	Eternal Standard, claim boundaries, workstream owners approved. Daily war room begins.	Day 3	Trust cleanup: Google profile, duplicates, old pages, public directories.
Day 4	Family and referral-facing message architecture drafted.	Day 5	Homepage, First 72 Hours page, For Families page, Referral page drafted.	Day 6	Phone scripts, intake checklist, referral script, claims boundary sheet drafted.
Day 7	Initial website copy approved; compliance-sensitive language routed for review.	Day 8	Referral one-sheet + Family Peace Guide outline.	Day 9	Hospice Decision Navigator drafted.
Day 10	Website page layouts and contact/referral forms built.	Day 11	CRM pipelines, automations, lead alerts, tags configured.	Day 12	Social profile copy and email signatures unified.
Day 13	Site + collateral reviewed for tone, trust, accuracy, conversion clarity.	Day 14	Internal team review: scripts, FAQs, intake flow, escalation. Daily war room ends.	Day 15	Inconsistencies corrected; soft launch package finalized. Weekly cadence begins.
Day 16	Forms, phone numbers, CRM alerts, autoresponders tested.	Day 17	Folder/card copy finalized; print direction selected.	Day 18	Print orders placed. Soft launch review list and feedback form ready.
Day 19	Soft launch: internal team + trusted advisors.	Day 20	Feedback collected; confusion, missing proof, risky language flagged.	Day 21	Revisions complete. Referral-ready version approved.
Day 22	Warm referral outreach begins with approved packet.	Day 23	Facility / physician / discharge-planner conversations scheduled.	Day 24	First family education post or resource published.
Day 25	Track Two assets prepared in parallel (separate plan): bio, fact sheet, quotes, brand story.	Day 26	Outreach continues; all activity logged in CRM.	Day 27	Inquiry/referral data reviewed; friction fixed inside 24 hours.
Day 28	Second educational resource published; referral follow-up sequence refined.	Day 29	First modality campaign template finalized.	Day 30	Thirty-day review: what launched, what converted, what confused, what to fix.



*This plan is built for a season of decision.
The brand is settled. The work ahead is
execution —
fast, careful, and laddered to The Eternal
Standard.*

*What follows is the steady, weekly act
of bringing every surface into parity
with what has already been decided.*

*Track Two follows on its own schedule —
on a foundation that this work makes
possible.*

*With love and care,
~ Conduit International*